The Access Statement

WORKING DRAFT v0.2 - NOT FOR FAMILIES - five items blocking publish

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The Access Statement - working draft, and what it still needs

Status: v0.2 WORKING DRAFT, 2026-09-26. **Not for families yet.** Part 1 is the family-facing text, assembled from commitments the design has already made, plus Erin's six decisions of 2026-09-26. Part 2 is what remains: three open questions, a site walk, one list that does not exist, and the counsel question this rides with. **Blocking before publish:** the site walk (Q5), the exceeds-capacity referral list (D), and the three open questions. **Why it exists:** Ring 3, Chekov. A real capacity limit exists and no line states it honestly, so it gets enforced invisibly as "not a fit," which is disability screening nobody decided to do. *"The learner who quietly could not participate never says no. They just don't come back. That silence is the barrier that hides best."* **Binding:** no first conversation runs before this is published. Chekov's condition, held in all three sittings. **Companions:** `2027-2028-launch-pad-first-cohort-conversation-v0.1.md` (the door), `launch-pad-the-familys-path-v0.1.md` (where this sits in the sequence), `2027-2028-launch-pad-coach-not-therapist-referral-pathway-v0.1.md` (the crisis roster, which is a different list).

The rule that governs every line

Describe the room, never the learner.

"There is no aide" is a fact about the room, and a family can decide what it means for them. "We cannot take a learner who needs an aide" is a fact about a learner, and it is the screening Chekov named. Every sentence below describes what is and is not in the room. None of them describes a kind of person.

The second rule follows from it: **this is read before the conversation, never during it.** A limit discovered in the room is a limit that has already done its damage.

Part 1 - The family-facing text (draft)

What this room can hold

The Launch Pad at Vibrant Life, 2027-2028

Before you decide whether this is right for your learner, here is an honest description of the room. We would rather you know now than find out in March.

Who is here

One guide, Erin. She holds a current CPR certification, a life coaching certification, and a degree in psychology. She is not a licensed clinician, and this is not therapy.

She will have a baby with her on guide days, worn while she works. There is no aide, no assistant, and no second teacher.

Tuesday and Thursday mornings she is in the room with the learners from 9:00 to 1:00. Monday and Wednesday mornings the learners run the morning themselves while she is on site in another room with the younger children, and she checks in around midday. Friday is not a studio day.

The room holds five to seven learners.

The space

`[PENDING SITE WALK — see Part 2, Q5. Do not publish this section until the room, the route in and the bathroom have been measured rather than remembered.]`

What the room provides

- **Anything written can also be said, drawn, typed, recorded, or told to someone who writes it down.** No version counts less than another. This holds for the Captain's Log, the hundred dreams, and every reflection all year.
- **Every circle has a way in that is not speaking:** a written card, an object, a drawing, or a pass with a nod. Passing is always allowed and never needs a reason.
- **Every "place yourself" activity has a seated version,** and every expedition has an equal alternative that is not a lesser version of it.
- **Handouts come in large print,** high contrast, left aligned, with starter banks instead of blank pages, and an icon version for anyone who freezes at a form.

- **Tools you can pick up and put down:** a timer, a template, someone to work alongside, a visible board showing where you are. Nobody is assigned these and nobody is tracked using them.
- **A quiet place to go, and standing permission to go there without asking.**
- **A shared meal on every guide day. Learners bring their own lunch; the studio does not provide the food.** We stop and eat together, the guide sits and eats with the learners, and nobody has to eat. Presence is the requirement, never consumption, and nobody watches anybody's plate. On the two days a year when there is shared food, the autumn potluck and the May feast, allergen, cultural and religious needs are planned in advance.
    - `INTERNAL, NOT FOR FAMILIES — split out with the others. The equity half of the Table floor is unbuilt as of 2026-10-19. It used to rest on a provided base meal and a no-cover-charge rule. Bring-your-own exposes a learner who cannot bring lunch, twice a week, at the belonging ritual. This statement must not publish until that is answered: a standing backstop on the table, or the honest blank. A door that only appears when someone needs it announces them for needing it.`
- **If being around people is genuinely hard,** the weekly accountability rhythm can run with the guide or on your own time instead of with a partner, and nobody reads that as failing to bond.
- **Pace is set by readiness, not by a date.** Scaffolding that stays in place all year is a real path, not a delay.
- **Nothing is graded, ranked, or compared,** and there are no deadlines a learner did not set.

What the room does not provide

- **No aide** or one-to-one support person.
- **No speech, occupational, physical, or behavioural therapy,** and nobody on site qualified to provide them.
- **No special education instruction, and no IEP or 504 plan.** We are not a school district and cannot write or carry out either one.
- **No evaluation or diagnosis** of anything.
- **No counselling or therapy.** Erin is a coach and a mentor, not a clinician. She does not diagnose and does not treat. If something comes up that is past coaching, her job is to stay with your learner and route to real help quickly, and she will tell you when that happens.

- **No nursing care.** Erin holds a current CPR certification. Beyond that there is no medical training here and no nurse on site. `[ERIN + COUNSEL — see Part 2, Q4: medication, and what medical plans one adult with an infant can honestly run]`
- **No physical intervention.** Nobody is restrained in this room. If someone is in danger, the plan is to get people to safety and call for help. That is a choice about how this room works, not a gap in it.
- **No adult in the room with the learners on Monday and Wednesday mornings.** Erin is on site and reachable, in another room, for about three hours. If your learner cannot hold three hours on their own, tell us and we will think it through together. Things we can talk about: a parent or another adult on site for those mornings, or a later start. Some situations we will not be able to solve, and we will say so plainly rather than let you find out in November.
- **Not an undivided adult on guide days.** There is a baby. The deep hour is protected and has a plan for when she needs tending, and some days it will still be interrupted.
- **One block of each guide day needs a room that can largely hold itself.** That block is the deep conversation. Occasional redirection is ordinary and fine. Continuous one-to-one attention is more than one adult can hold while also holding the hour.

What to tell us

We are not asking for a diagnosis, and we do not need one. We are asking what shapes whether this particular room works for your learner. Useful things to tell us:

- What helps them learn, and what gets in the way
- How they like to get their thoughts down
- What would make this room hard: noise, light, food, sitting still, being read aloud to, being looked at
- Anything about three self-managed hours on Monday and Wednesday we should think through together
- Anything medical that would need a plan
- Anything about a daily shared meal we should know

If this is not the right room

Sometimes the honest answer is that we cannot hold what your learner needs. If that happens we will tell you plainly, in writing, and we will tell you why. Then we will point you somewhere real. We will not say “not a fit” and leave you to guess.

`[ERIN — see Part 2, item D: the named list of where we point people]`

Part 2 - What this still needs

A. The nine judgment calls

Six settled with Erin on 2026-09-26. Three remain, and none of the three can be settled by her alone.

Settled

#	The call	What Part 1 now says
Q1	**Three unsupervised hours.** Not a flat structural limit. Tell us and we plan it together	The three hours are stated as fact, followed by an invitation to raise it and two concrete things that can be talked about (an adult on site those mornings, a later start), plus the honest line that some situations will not be solvable and we will say so rather than let a family find out in November. **The named examples exist because a private case-by-case judgment with no written criteria is where invisible screening lives**
Q3	**Redirection.** Occasional yes, continuous no	Written as a fact about the hour, not about a learner: one block of each guide day needs a room that can largely hold itself
Q5	**The building.** Needs measuring, not remembering	Section held with a `[PENDING SITE WALK]` marker. Checklist below. Do not publish with it unfilled

#	The call	What Part 1 now says
Q7	**Behaviour.** Describe the room; the credentials move up front	Erin's CPR certification, life coaching certification and psychology degree are named in *Who is here*, with "not a licensed clinician" in the same breath, because credentials sitting beside a crisis paragraph read as clinical capacity and that is the misread the referral pathway exists to prevent. The behaviour line itself is now: nobody is restrained here, and if someone is in danger the plan is to get people to safety and call. **A choice about how the room works, not a gap in it**
Q8	**If Erin is out for a day.** Out of this document	Moves to the Family note. Keeps the access statement about access specifically
Q9	**Whose statement.** Launch Pad now, flagged as precedent	Published for the studio. **Jenna and Wes are told plainly, before it goes out, that it will read as the school's template**, because there is no school-wide access statement anywhere in Vibrant Life today

Still open

#	The question	Who has to be in it
Q2	**The Table.** A shared meal every guide day, with a learner in active eating-disorder recovery. Can the room hold that?	**The clinician, not only Erin.** The inclusion floor already says presence not consumption and no plate is watched, which is good design but is not a clinical judgment. Salus's HOLD-FOR-HUMAN on Wellness is the adjacent precedent

#	The question	Who has to be in it
Q4	**Medical.** Medication administration, a seizure disorder, Type 1 diabetes, an allergy needing epinephrine. What can one adult with an infant and a CPR certification honestly run?	**Counsel, plus Erin.** Rides with the C bundle below
Q6	**Emerging English.** Origin is a writing-heavy season. Multi-modal capture is already law, which helps. Is that enough?	**Erin, with Kira's read.** Boise has refugee resettlement families, and this is where the plurality finding lands

The site walk (Q5) - measure, do not remember

- Doorway widths: the room, and the bathroom
- A step-free route from drop-off to the room, including any threshold strips
- Bathroom: same floor, turning space, grab bars, door that can be managed one-handed
- Inside the room: can a wheelchair reach the circle, the Table, and the agreements wall
- Light: can it be dimmed, and is it fluorescent
- Sound: what is adjacent on Tue and Thu mornings, and can it be quieted
- The quiet space: where it is, whether it can be reached without asking, and whether it is accessible
- The exit route in an emergency, by the same test

B. The dependency nobody has named

The crisis roster and the exceeds-capacity list are two different lists, and only one exists.

The referral pathway has a real roster already built: 988, Crisis Text Line, Clarvida Youth Support Center in Boise, Idaho DHW mandatory reporting, 2-1-1, SAMHSA. Real numbers, current, good work. It has exactly one blank, the named warm-referral therapist, which is P0-1.

But that roster is for **crisis routing**, and the access statement needs a different one: **where a family goes when the honest answer is that this room cannot hold what their learner needs.** That is not a crisis. It is "this is not the right room, and here is a room that might be." Nothing in any document names those places.

Without it, the last section of the statement is a promise we cannot keep, and the whole document reverts to "not a fit," which is the thing it exists to prevent.

C. The counsel question it rides with

Do not send this to counsel alone. It belongs in the same conversation as the duty-of-care question the year plan already flagged: on Monday and Wednesday mornings, learners aged sixteen to eighteen are self-managing while Erin is on site but in the Tots room. Is “on site and reachable” sufficient under Idaho and umbrella-school rules, or must an adult be in the room?

Q1 and Q4 above are the same conversation. One engagement, three answers. This is G1, the long pole, and the one gate the fleet cannot self-certify.

D. The one list to build

Boise-area places to point a family when the answer is no. Real names and current numbers, the same standard the crisis roster already meets. Candidates worth calling: the district’s special education office, homeschool co-ops with aide support, private programs with special education staffing, a special education advocate, the state parent training and information centre, the library’s accessibility services.

Small research task. Nothing blocks starting it today.

E. The reads

- **Chekov, Tier 1.** He named the finding and holds the read on the finished statement.
- **Kira.** Plain language at eighth grade, interpreter offered.
- **Accord.** The Exhale Test on the “what the room does not provide” section and on the “not the right room” letter, which is the same family reading both.
- **Comes.** Reads it as the sixteen-year-old, not only as the parent. The learner reads this too.

F. Sequence

1. ~~Erin answers the nine~~ **six done 2026-09-26.** Q2, Q4 and Q6 remain
2. Walk the building with the checklist above and fill *The space*
3. Build the list in D, and make the calls for the P0-1 clinician in the same week, since they are the same kind of phone call. **Q2 is answered in that same clinician conversation**
4. Send the statement to counsel bundled with the Monday and Wednesday duty-of-care question, which also answers Q4
5. Chekov, Kira, Accord and Comes read. **Q6 is answered inside Kira’s read**

6. Jenna and Wes see it before it publishes, and are told plainly it will read as the school's template
7. Publish, at Stage 0, before any letter goes out and before any conversation happens

Note the shape: each of the three open questions is answered inside a step that was already owed. None of them needs its own meeting.

Decisions of 2026-09-26 (for the record)

Erin settled six of the nine in conversation. The two worth carrying forward as reasoning, not just as answers:

On Monday and Wednesday, she chose the open door over the clean limit. The warmer choice and probably the right one, but it costs a private judgment every time a family raises it, and a private judgment with no written criteria is the invisible screening this document exists to prevent. Hence the named example plans in Part 1. The examples are the criteria, made public.

On behaviour, she pushed back on the draft and was right. The original line read "nobody trained in physical intervention or crisis de-escalation," which describes a lack rather than a room, and understated a guide who holds CPR, a coaching certification and a psychology degree. Two fixes came out of it. The credentials moved to *Who is here*, because a psychology degree sitting beside a crisis paragraph reads as clinical capacity and the whole referral pathway exists to prevent that misread. And the behaviour line became a description of what the room does instead: nobody is restrained, people get to safety, someone calls.

The test she and the room used for any future training: **training that makes the routing faster and surer is aligned; training that makes the guide hold more clinical weight herself is drift**, because the entire safety floor rests on the coach-not-therapist boundary staying bright. By that test, Youth Mental Health First Aid and QPR sharpen the job the design already gives her. The physical-restraint half of crisis-intervention training moves the other way, against a program built on consent and the right to decline. *(Availability and current content of any named course need checking before anyone books one.)*

A limit written down is care. A limit enforced without being written down is screening. This document is the difference between the two.

"We evoke - we never extract."